



Netramind

STANDARD OPERATING PROCEDURE

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Title: Clinical Data Transfers

1. PURPOSE

This SOP describes the process for managing the secure transfer, validation, and handling of clinical data between Netramind and external parties in support of clinical trials.

2. SCOPE

This procedure applies to clinical data transfers between Netramind and external parties, including data received from clinical sites, imaging centers, and sponsors, as well as data provided by Netramind (e.g., image-derived outputs, grading results, and datasets).

3. REFERENCES

- ICH E6(R3): Guidelines for Good Clinical Practice (GCP)
- 21 CFR Part 11 Electronic Records and Electronic Signatures

4. DEFINITIONS

- 4.1. Audit Trails – Metadata records that allow the appropriate evaluation of the course of events by capturing details on actions (manual or automated) performed relating to information and data collection and, where applicable, to activities in computerized systems. The audit trail should show activities, initial entry, and changes to data fields or records, by whom, when and, where applicable, why. In computerized systems, the audit trail should be secure, computer-generated, and time stamped.
- 4.2. Clinical Data Transfer—A transfer of data which will be used for clinical trial operations, analysis, or potential regulatory submission. Data transfers can be cumulative (all data) or interim (new data or targeted data), according to the data specification plan. Examples of data transfers include:
 - Routine Data Transfer—A transfer of data which will be used for the purposes of data review, reconciliation, and other ongoing, iterative data quality activities.
 - Final Data Transfer—A transfer clinical study data which have been cleaned for database lock or data for an interim lock.
- 4.3. Contract Service Provider (CSP)— A person or organization (commercial, academic or other) providing a service used during the conduct of a clinical trial to either the sponsor or the investigator to fulfil one or more of their trial-related activities.
- 4.4. Data Acquisition Tool (DAT) – Paper or electronic tool designed to collect data and associated metadata from a data originator according to the protocol and to report the data to the sponsor. Data originator may be a person (e.g., participant or trial staff), a device (e.g., wearables and sensors) or an electronic transfer of data from one system to another (e.g., extraction of data from an electronic health record or laboratory system). Examples of DAT



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include but are not limited to case report forms (CRFs), interactive response technologies (IRTs), patient-reported outcomes (PROs), clinical outcome assessments (COAs) and wearable devices, irrespective of the media used.

- 4.5. Data Transfer Specification (DST) Plan or Agreement —A document agreed to by both Netramind and the data service provider that describes the content and format of the data to be transferred as well as the method of transfer. Data Transfer Specifications are known by other names including Data Transfer Agreements and Data Transfer Plans.

5. ROLES AND RESPONSIBILITIES

5.1. Data Management

- 5.1.1. Work with the CSP to finalize and approve Data Transfer Specifications (DTS) for clinical data within scope of this SOP.
- 5.1.2. Test data transfers and documenting the outcome.
- 5.1.3. Perform validation checks on data transfers to ensure adherence with DTS.
- 5.1.4. Review data transfers to ensure adherence to DTS complete and accurate data has been transferred. Releasing the data for use.
- 5.1.5. Provide transferred files to Statistical Programming when the transfer is not automated.
- 5.1.6. Perform periodic reviews of audit trails and relevant metadata as part of data integrity checks.

5.2. Statistical Programming

- 5.2.1. Review Data Transfer Specifications to ensure appropriate analysis and reporting requirements are included.
- 5.2.2. Perform validation checks on data transfers to ensure adherence to DTS.

5.3. Biostatistics

- 5.3.1. Review Data Transfer Specifications to ensure appropriate analysis and reporting requirements are included.
- 5.3.2. Review final and critical data transfers for adherence to DTS.
- 5.3.3. Accept final data transfers for analysis purposes.

6. PROCEDURE

6.1. Creating Data Transfer Specifications

- 6.1.1. A Data Transfer Specification (DTS) Plan should be created where structured or recurring data transfers are expected.



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- 6.1.2. For Netramind, data may include ophthalmic images, derived measurements, grading outputs, adjudication decisions, and associated metadata.
- 6.1.3. Data Management or Statistical Programming initiates a data transfer specification with CSP, typically using the provider's template.
- 6.1.4. The Data Manager ensures the template includes:
 - Relevant data acquisition tool (DAT) and systems data is collected and processed
 - Source records
 - Validation requirement and status
 - Variable specifications
 - Protection of data that can unblind (if any)
 - Secure, access-controlled transfer methods (e.g., sFTP, secure cloud storage with controlled access)
 - Transfer format per agreement with Statistical Programming and Biostatistics
 - Frequency, scheduled interims and types of transfer during the data lifecycle of the clinical trial.
 - Expectation for quality control of data transfers performed by CSP and/or Sponsor.
- 6.1.5. The Data Manager coordinates review of the DTS Plan with relevant stakeholders (e.g., Biostatistics, Statistical Programming). Final approval is documented once all comments are resolved.
- 6.1.6. The Data Manager manages change control throughout the data lifecycle should changes to data structure/specifications or other transfer requirements by ensuring that the DTS Plan is revised, reviewed, and approved according to this SOP.

6.2. Verifying Test Data Transfers

- 6.2.1. For data from the EDC study database
 - 6.2.1.1. Data Managers test data extraction during user acceptance testing (UAT) of the database if the EDC provides that capability. Otherwise, the database specifications document, which is tested in UAT against the entry screens, provides adequate assurance of the upcoming extracts, with the EDC and its specifications upload process being validated software.
- 6.2.2. For other types of data (non-EDC)
 - 6.2.2.1. The Data Manager oversees an initial test transfer from the CSP before the data collection process begins and confirms that all key elements of the specification including file names, file format, variables, and blinding are correct according to the DTA.



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6.2.2.2. The Data Manager signs the document to confirm that a test transfer was performed and was successful.

6.2.3. If the DST plan or agreement is updated to reflect changes in the data transferred or file contents and structure, the Data Manager must conduct and document a new test of the transfer. Administrative changes such as updates to contact information do not require a retest.

6.2.4. Test transfers should confirm file structure, completeness, and compatibility with downstream systems.

6.3. Managing Data Transfers

6.3.1. The Data Manager receives the data transfer(s) via a secure transfer method and upload the files on the Netramind dedicated and secure server if an automated transfer via secure method is not available.

6.3.2. For routine data transfers, a Data Manager or Statistical Programmer initiates checking of the transfer, which includes confirming that the number of files and variables are consistent with previous transfers and that the number of data records is the same or increasing for each file. Routine transfers should be logged or traceable, even if detailed review is not required.

6.3.3. For critical or final transfers, validation checks should confirm data completeness, consistency, and expected changes relative to prior transfers.

6.3.3.1. The Data Manager reviews the comparison output and researches any differences shown. This can be verified by reviewing the metadata and/or audit trails generated when changes are made in a system. The changes must be valid and acceptable according with study plans and decisions.

6.3.3.2. If all differences are found to be valid, the Data Manager signs the comparison output to indicate that the data is complete and accurate. The Data Manager files the signed output document in the TMF.

6.3.3.3. If issues are detected, the Data Manager requests a new transfer from the CSP and works with the CSP until all issues are resolved.

6.3.3.4. The Data Manager notifies the study team when the data is considered available for use.

6.3.3.5. The Biostatistician confirms that the transferred data satisfy the quality required for analysis, and which data is required for review and retention.

6.3.4. For data provided by Netramind to external parties, verification must be performed to ensure completeness, accuracy, and alignment with agreed specifications prior to release.



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6.4. Filing and Archival of Data Transfers

- 6.4.1. All documentation related to data transfers, including the actual records, data transfer files must be filed and maintained in accordance with SOP-ABC-123 GxP Record Management procedures.

7. ATTACHMENTS

- Appendix I: Workflow for Data Transfers



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8. REVISION HISTORY

VERSION NUMBER	Date	Summary of Change(s)
01	01-MAY-2026	New Document
[X.0]	[DD-MMM-YYYY]	[Insert a summary of the changes]



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SIGNATURE PAGE

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APPROVER

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Workflow for Data Transfers

